

The Episode, End to End

Read beside Primary-Flow-Map — same blocks, same positions. The tool shows; it does not act

- Intake

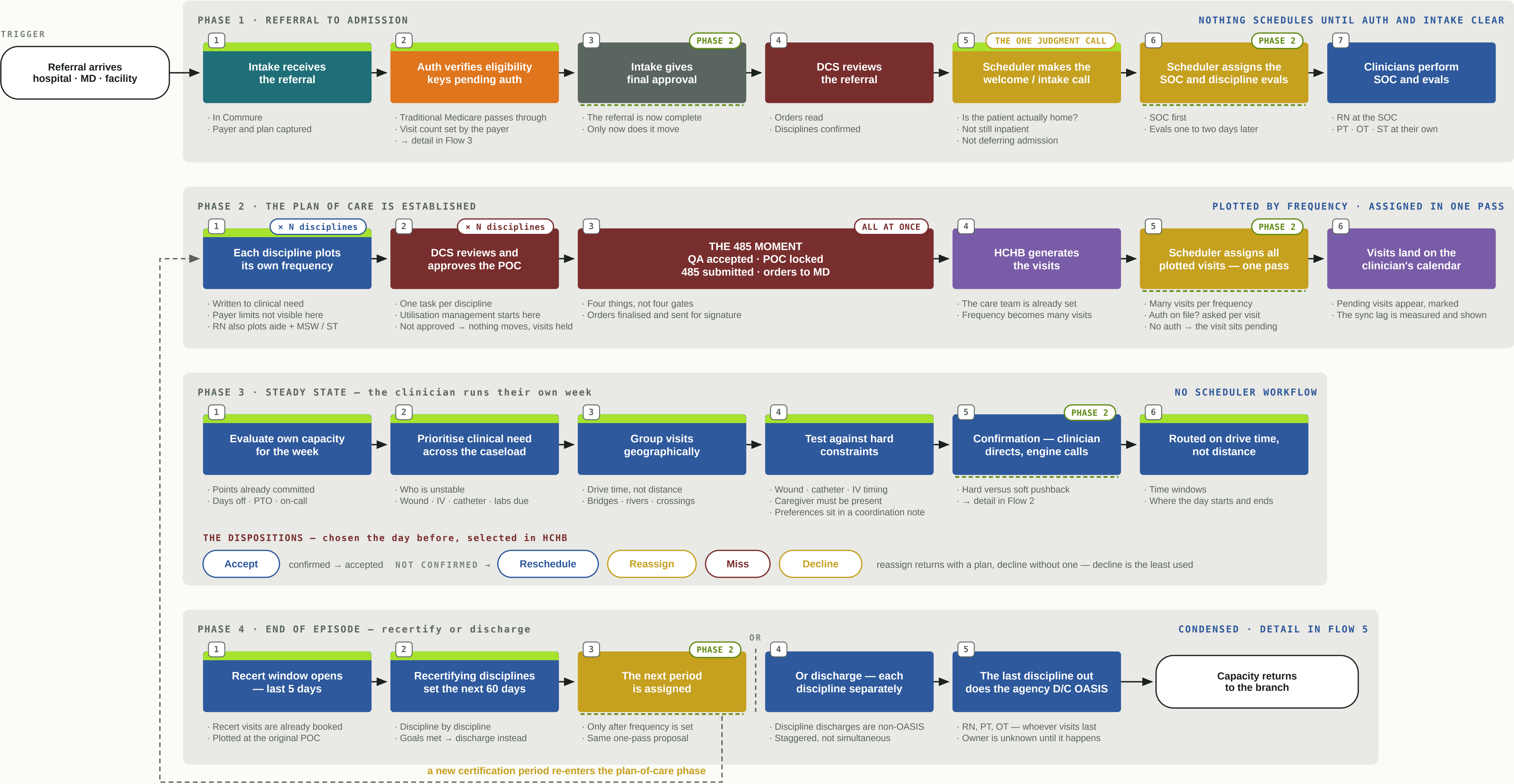
Clinician

Shown by the tool
- Insurance & Auth

HCHB
- PCC / Scheduler

Per Diem / Float
- DCS

Branch Leadership



PHASE 2 · THE PLAN OF CARE IS ESTABLISHED

1

× N disciplines

Each discipline plots its own frequency

· Written to clinical need

· Payer limits not visible here

· RN also plots aide + MSW / ST

2

× N disciplines

DCS reviews and approves the POC

· One task per discipline

· Utilisation management starts here

· Not approved → nothing moves, visits held

3

THE 485 MOMENT

QA accepted · POC locked

485 submitted · orders to MD

· Four things, not four gates

· Orders finalised and sent for signature

4

HCHB generates the visits

· The care team is already set

· Frequency becomes many visits

5

PHASE 2

Scheduler assigns all plotted visits — one pass

· Many visits per frequency

· Auth on file? asked per visit

· No auth → the visit sits pending

6

Visits land on the clinician's calendar

· Pending visits appear, marked

· The sync lag is measured and shown

PLOTTED BY FREQUENCY · ASSIGNED IN ONE PASS

PHASE 3 · STEADY STATE — the clinician runs their own week

1

Evaluate own capacity for the week

· Points already committed

· Days off · PTO · on-call

2

Prioritise clinical need across the caseload

· Who is unstable

· Wound · IV · catheter · labs due

3

Group visits geographically

· Drive time, not distance

· Bridges · rivers · crossings

4

Test against hard constraints

· Wound · catheter · IV timing

· Caregiver must be present

· Preferences sit in a coordination note

5

PHASE 2

Confirmation — clinician directs, engine calls

· Hard versus soft pushback

→ detail in Flow 2

6

NO SCHEDULER WORKFLOW

Routed on drive time, not distance

· Time windows

· Where the day starts and ends

THE DISPOSITIONS — chosen the day before, selected in HCHB

Accept

confirmed → accepted

NOT CONFIRMED →

Reschedule

Reassign

Miss

Decline

reassign returns with a plan, decline without one — decline is the least used

PHASE 4 · END OF EPISODE — recertify or discharge

1

Recert window opens — last 5 days

· Recert visits are already booked

· Plotted at the original POC

2

Recertifying disciplines set the next 60 days

· Discipline by discipline

· Goals met → discharge instead

3

PHASE 2

The next period is assigned

· Only after frequency is set

· Same one-pass proposal

4

OR

Or discharge — each discipline separately

· Discipline discharges are non-OASIS

· Staggered, not simultaneous

5

The last discipline out does the agency D/C OASIS

· RN, PT, OT — whoever visits last

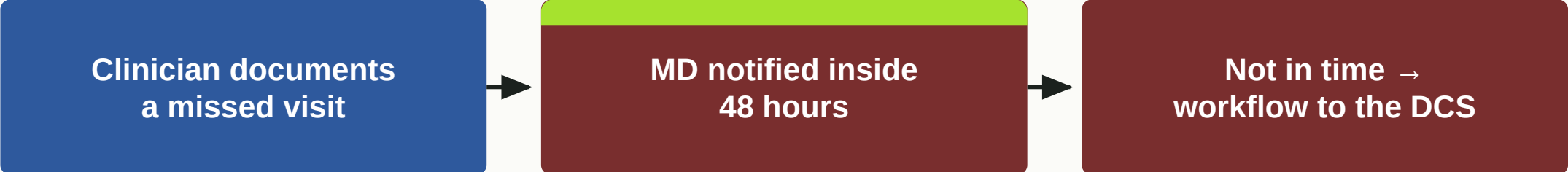
· Owner is unknown until it happens

Capacity returns to the branch

CONDENSED · DETAIL IN FLOW 5

a new certification period re-enters the plan-of-care phase

MISSED VISIT AND LEVERS



A compliance chain, not a dead end — Medicare requirement and an HCHB hard stop

PULLED ON PURPOSE WHEN CAPACITY TIGHTENS — not recovery, instrument



- Takes the SOC's, absorbing the admission spike
- Or covers visits to free a territory clinician

- Territory alignment
- Referral acceptance when capacity tightens

READING THIS MAP

THREE DIFFERENT CEILINGS ON ONE EPISODE

- Auth is permission — how many visits the payer will allow
- LUPA is the floor — too few visits and the period pays per visit
- Utilisation management is the ceiling — extra visits earn nothing

WHAT THIS MAP IS

- Target state — every block sits where the current-state map put it
- Green is the engine. Purple is still HCHB. A colour bar means a person decides
- Numbered chips read against the same numbers on Primary-Flow-Map

WHERE IT BROKE



ADDRESSED ABOVE

ADDRESSED ABOVE